Behavioral Health Agency Inspection Report

Department of Health
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February 24, 2020

Sea Mar Renacer Treatment Center, 10001 17th Pl S, Seattle, WA. 98168-1615	Kristen Prentice
Agency Name and Address	Administrator

ONGOING - ROUTINE	10/22/19	Jennifer M Ross LMHC
Inspection Type	Inspection Onsite Dates	Inspector
X2019-469	RTF.FS.00001053	
Inspection Number	License Number	

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the on-site inspection.

Deficiency Number and Rule Reference	Observation Findings	Plan of Correction
1015 Resident rights WAC 246-337-075(4)(a) The licensee must establish a process to ensure resident rights are protected in compliance with chapter 71.12 RCW, this chapter, and other applicable laws, and are based on the service types provided in the RTF. This process must address how the RTF will:(4) Protect the confidentiality of: (a) Treatment and personal information when communicating with individuals not associated or listed in the resident's individual service plan or confidentiality disclosure form;	Washington Administrative Code was not met as evidenced by: Based on clinical record review, the RTF did not protect the confidentiality of the treatment and personal information when communicating with individuals not associated or listed in the resident's individual service plan or confidentiality disclosure Failure to protect confidentiality of residents may result in a information being released without resident's permission and /or a fine due to release of HIPPA information. Findings include: 1. On 10/22/19, in review of clinical records it was determined that Records #2 and 3 did not have proper releases of information completed in the file for the persons that the facility was speaking with. This also includes probation reports that were being	The clinical staff was re-trained on the importance of ROI, 42 CFR Part 2, and allowable exclusions for confidentiality. The staff member responsible for these charts no longer works at Sea Mar. Monthly clinical chart audits will be conducted for 6 months to ensure ongoing compliance.

	sent without proper release of information 2. On 10/22/19, in an interview with Staff #2, Program Director, it was confirmed that neither file had the appropriate release of information for the information that was being communicated 3.	
1060 Resident care services WAC 246-337-080 (2)(b) (2) The licensee must:(b) Conduct or accept a current health care screening of each resident upon admission including a tuberculosis risk assessment and symptom screening;	Washington Administrative Code was not met as evidenced by: Based on clinical record review the RTF did not ensure that a current health care screening of each resident including a TB risk assessment and symptom screening was being completed Failure to have this health care screening may result in the facility not knowing of a resident with TB and being unable to provide the care needed as well as putting other staff/residents at risk of TB. Findings include: 1. On 10/22/19, in a review of clinical records it was determined that Records #1/2/3 did not have a health care screening which included a TB risk assessment and symptom screening was being completed	To correct this finding, all youth entering treatment will complete a TB symptom screening on paper (see attached) that will be scanned into the electronic health record. If a youth screens, high-risk they will be referred to the medical clinic for further screening. Sea Mar is slated to change electronic health record systems in 2020 and in that system, the TB screener questions are included in the admission documentation requirements.
1085 Resident care services WAC 246-337-080(2)(g) (2) The licensee must:(g) Provide access to tuberculosis testing if the resident is high-risk or symptomatic of tuberculosis;	The Washington Administrative Code was not met as evidence by: Based on clinical record review, policy review and interview with staff, it was determined that the clinical record did not document that a TB test was complete. Failure to meet this standard may result in substandard care for patients and not meeting the health needs of all patients. Findings include: 1. On 10/22/19, in a clinical record review, it was determined that Records #1/2/3 did not have a TB test completed on residents.	Based on our records, there is no evidence that these patients were symptomatic or high-risk therefore a TB test was not required. A meeting will be set up between our Adolescent clinic physician/staff to determine the best process for getting the TB testing results transferred to Renacer following physical exams. Each youth will submit to a TB blood test if high risk/symptomatic. The results will be printed and provided to Renacer staff at our next clinic day. The results will be kept in the MAR binder as well as scanned into the EHR. As per policy, all

		youth are required to have a physical exam completed within 5 days of admission.
1150 Resident care services WAC 246-337-080(5)(a) (5) RTFs which meet the conditions in subsection (3) or (4) of this section must:(a) Perform a health assessment for each resident. A prescriber or licensed nurse operating within their scope of practice shall conduct and complete the assessment following the resident's admission to the RTF unless a health assessment was performed within the past three months and is available to the RTF upon admission; and	The Washington Administrative Code was not met as evidence by: Based on clinical record review, policy review and interview with staff, it was determined that the clinical record did not document that a health assessment was performed on each resident. Failure to meet this standard may result in substandard care for patients and not meeting the health needs of all patients. Findings include: 1. On 10/22/19, in a clinical record review, it was determined that Records #1/2/3 did not have a health assessment performed on each resident.	The names of these patients was requested and we have not received a response yet. If the document is available in the record, we can submit it for review. There is a possibility that record of the physical exam was in the medication binder and not yet scanned into the EHR. It is Sea Mar policy that every youth attends a physical exam within the first 5 days of their stay.
1385 Resident's individual service plan WAC 246-337-100(1)(a-b) (1) The licensee must develop and implement an individual service plan for each resident based on the resident's: (a) Initial health on admission; and (b) Health assessment(s).	The Washington Administrative Code was not met as evidence by: Based on clinical record review, policy review and interview with staff, it was determined that the clinical record did not document that a health assessment was performed on each resident that could be included on the service plan Failure to meet this standard may result in substandard care for patients and not meeting the health needs of all patients. Findings include: 1. On 10/22/19, in a clinical record review, it was determined that Records #1/2/3 did not have a health assessment performed on each resident so it could be included on each service plan.	An in depth staff training on individual service plans will be conducted on 3/11/2020 and will cover the need to have health needs included in the ISP. Monthly clinical chart audits will be conducted for 6 months to prove compliance with this requirement.
1400 Resident's individual service plan WAC 246-337-100(2)(c) (2) Individual service plans must:(c) Include work assignments given to a resident	Washington Administrative Code was not met as evidenced by:	An in depth staff training on individual service plans will be conducted on 3/11/2020 and will cover the need to

as part of their individual service plan, if applicable;	Based on clinical record review, it was determined that the clinical record did not include work assignments given to residents as part of their individual service plan. Failure to include work assignments as part of their service plan may result in resident's work not being acknowledged properly Findings include: 1. On 10/22/19, in a clinical record review, it was determined that Records #1/2/3 did not have service plans that reflected the work assignments given to residents including: responsible for the cleaning of entire facility and responsible for maintaining clean room. 2. On 10/22/19, in interview with Staff #2, Program Director, it was determined that these items were not put on their service plan but they were part of the resident's assigned duties	include community engagement/ cleaning tasks in the ISP. Monthly clinical chart audits will be conducted for 6 months to prove compliance with this requirement.
1405 Resident's individual service plan WAC 246-337-100(2)(d) (2) Individual service plans must:(d) Be updated as additional needs are identified during treatment; and	Washington Administrative Code was not met as evidenced by: Based on clinical record review, it was determined that service plans were not updated to reflect assignments that were given to residents Failure to update service plans may result in residents not meeting all their service plan goals Findings include: 1. On 10/22/19, in a review of resident records, it was determined that Record #3 did not reflect assignments given to resident as part of their treatment	An in depth staff training on individual service plans will be conducted on 3/11/2020 and will cover the need to include assignments assigned to clients in the ISP and the progress toward those assignments in the ISP review. Monthly clinical chart audits will be conducted for 6 months to prove compliance with this requirement.
1410 Resident's individual service plan WAC 246-337-100(2)(e) (2) Individual service plans must:(e) Include a discharge health care plan.	Washington Administrative Code was not met as evidenced by: Based on clinical record review, it was determined that the service plans did not include a discharge health care plan	An in depth staff training on individual service plans will be conducted on 3/11/2020 and will cover the need to include discharge planning tasks in the ISP. Monthly clinical chart audits will

	Failure to have discharge health care plans may result in the resident's health care needs not being met Findings include: 1. On 10/22/19, in review of clinical records, it was determined that Records #1/2/3 did not have discharge health care plans reflected on the service plan.	be conducted for 6 months to prove compliance with this requirement.
1980 Facility and environment requirements WAC 246-337-120(1) (1) The licensee must maintain the facility, exterior grounds, and component parts such as fences, equipment, outbuildings, and landscape items in a manner that is safe, free of hazards, clean, and in good repair.	Washington Administrative Code was not met as evidenced by: Based on the inspection of all areas of the facility, the RTF failed to ensure that the facility was safe, sanitary, and being maintained in good repair. Failure to keep the facility maintained in good repair may result in poor hygiene, bugs and rodents, mold growth, health concerns to residents, overall unsanitary condition. Findings included: 1. During the survey on 10/22/19, at 11:30 am the surveyor observed the following; a) The shower room for all residents was in need of repair, the tile was broken off the walls and flooring area exposing the wood and metal material. See pictures attached (#13, #18, #19). b) The shower room ceiling showed an unfinished drywall job centered around an open light fixture with no cover. See picture attached (#16). c) Loose mop boards need to be secured to the wall in the main hallway where resident's rooms are located. See picture attached (#10). d) The floors in the hallway to the resident's bedrooms were damaged coming apart at the joints. The damage to the floors created rough edges for dirt and mold to catch in and produce mold, and bacteria. The flooring is also a health and tripping hazard. This surveyor tripped on the floor when walking in the hallway. See pictures attached (#8, #9). e) Large holes in walls of resident's bedrooms. See	a. The broken tile and flooring was corrected in November by the maintenance person at the building. b. The drywall patch in the bathroom was repaired on 3/4/2020. c. The mop boards at the base of the wall will be secured to the wall by 3/11/2020. d. We have begun construction on a portion of our building. We are attempting to salvage the flooring from that portion of the building to repair the flooring on the Inpatient wing. If salvage is not possible, we will forgo this repair as the flooring is going to be replaced fully during construction. If possible, this work is slated to be completed by 3/25/2020. e. The holes in resident rooms are repaired on a weekly basis. The holes noted in the audit have been repaired. f. The bathroom identified is a single occupancy shower room. The bathroom doors lock automatically and entry is controlled by staff. The bathrooms will be demolished as part of construction. g. The graveyard staff have begun cleaning the bathroom nightly, in addition to the youth's daily chores. This has ensured that the bathroom is

	pictures attached (#3, #6). f) No Shower curtains for privacy in the large shower room. g) Small bathroom with bathtub was dirty with mold and grime. See pictures attached (#13, #14). h) Two toilets for 16 male clients, one toilet was missing the lid the other toilet had the lid in place. i) The Medication Room had ants crawling on the wall and counter top and around sink. 2. During an interview with staff member #A, on 10/22/19, at 11:30am, in reference to the above deficiencies, he stated that the reason the one toilet was missing the lid was due to residents using the lid as a weapon. When asked about the holes in the walls he stated that the residents destroy things so fast that it is hard to keep up with the damage. During the interview the question was brought up about the rope used to hold the curtains up and if he saw it as a hazardous situation the staff stated that it was not a problem. When asked why a rope is used instead of a curtain rod the staff did not have a reason for no curtain rod. This surveyor was concerned that the staff member stated that the reason the one toilet lid was removed was that it could be used as a weapon, but the second toilet lid was not removed. The observation for this surveyor was the absence of continuous and consistent safety in the facility for residents.	kept cleaner than in the past. Additionally, staff is responsible for touch up cleaning at least one time on their shift. h. The toilet in the small bathroom was replaced in February and it currently has the tank lid attached. i. The exterminator had just been to Renacer prior to the site visit and the ants were crawling to the bait on the wall. Renacer has monthly scheduled extermination services as well as "as needed" service. The exterminator returned to the site the day after the site visit and he moved the bait placement so that the ants were not crawling on the counter. We will continue to monitor the pest control situation. A running maintenance log will be created that lists deficiencies, date added and date completed. This will be kept in the manager's office to ensure maintenance is being completed. Construction has begun to renovate the building and keeping up with repairs will be a priority.
2280 Resident sleeping room requirements WAC 246-337-129(1) (1) The licensee shall provide residents with an accessible, clean, well-maintained room with sufficient space, light, and comfortable furnishings for sleeping and personal activities.	Washington Administrative Code was not met as evidenced by: Based on the survey of the resident rooms, the RTF failed to provide residents with an accessible, clean, well-maintained rooms for sleeping and personal activities and security of their personal belongings. Failure to provide residents with accessibility to well maintain residence rooms, with comfortable furnishings for sleep and personal activities may result in residents not having a clean	Bedroom doors are locked due to safety and supervision concerns as Rencer is a Level 2 secure facility. We will install hooks next to each bedroom door for the youth's sweat-clothes when they are not wearing them. To be completed 3/18/2020. We have a binder rack in the hall to

	and comfortable space to rest and store their personal belongings in a hygienic space. Findings include: 1. During the survey of the RTF, the surveyor found the residents rooms not maintained and not accessible to residents. The residents bedrooms are kept locked through the day and the residents were only allowed in their rooms to collect items, come back out and the doors were locked again by staff. At one point the residents were getting ready to work outside in rain gear and boots and had to leave their tennis shoes and shirts and sweatshirts on the dirty hallway floor outside their bedrooms due to no staff availability to unlock their bedroom doors so they could secure their personal belongings in their rooms.	provide a place for each youth to store their binders and papers throughout the day. Their slippers and slides will be placed in the shoe closet when they are not being used throughout the day and in their rooms at night. Their shoes are to be stored in the shoe closet at all times when not being worn. Construction has begun to renovate the building and there are plans for there to be cubbies or shelves placed next to the youth's rooms to store their items throughout the day.
2305 Resident sleeping room requirements WAC 246-337-129(2)(e)(i) (2) Sleeping rooms must include:(e) One or more outside windows that:(i) Has adjustable curtains, shades, blinds, or equivalent installed at the windows for visual privacy;	Washington Administrative Code was not met as evidenced by: Based on the survey of the resident rooms, the RTF failed to provide residents with adjustable curtains, shades, blinds, or equivalent installed at the windows or visual privacy. Failure to provide residents with adjustable curtains, shades, blinds, or equivalent installed at the windows or visual privacy may result in residents not having safe, well maintained rooms with privacy. Findings include: 1. During the survey of the RTF, the surveyor found the residents rooms had curtains strung on a ropes held up by screws used as a curtain rods in each resident's rooms. This reviewer asked (Staff# A Camron Hargrove) if this was a concern for possible harm to residents, he was unsure. I shared that it was a strong rope secured well and could be taken apart and used to harm self. This did not appear to be a concern to staff #A member.	New blinds were ordered on 2/13/2020 and were installed on 2/27/2020. Surplus blinds have been procured to ensure that if damage occurs to one set, there are replacements available.
2325 Resident sleeping room requirements	Washington Administrative Code was not met as evidenced	Renacer washes towels and washcloths

WAC 246-337-129(3)(b) (3) Sleeping rooms must be equipped with:(b) An individual towel and washcloth rack or an equivalent method to provide clean towels and washcloths; and	by: Based on the survey of the resident rooms, the RTF failed to provide residents with sleeping rooms equipped with an individual towel and washcloth rack or an equivalent method to provide clean and dry towels and washcloths. Failure to provide resident rooms equipped with towel and washcloth racks may result in residents getting bacteria, viruses or rashes due to unclean damp towels or washcloths. Findings include: 1. During the survey of the RTF on 10/22/19 at 12:30, the surveyor found the resident rooms did not have a place to hang towels and washcloths for their use.	after each use. The youth put the used towel in the laundry after their shower/hygiene routine. They are then washed, dried and given to the youth at their next shower period. No towels or washcloths are stored in the bedrooms.
2375 Lighting emergency light, electric outlts WAC 246-337-140(1) The licensee shall ensure that lighting, emergency lighting, and electrical outlets are adequate and safe including:(1) Protection of bulbs and tubes against breakage by using canned lights, appropriately fitted shields, or shatter resistant materials in all areas occupied by residents, and in medication and food preparation areas.	Washington Administrative Code was not met as evidenced by: Based on the inspection of all areas of the facility, the RTF failed to ensure that lighting, was adequate and safe. The RTF failed to provide protected covers for bulbs and tubes appropriately fitted shields or shatter resistant materials in all areas occupied by residents. Failure to repair broken light covers and shields may result in residents getting hurt by shattered glass from unprotected light bulbs and or electrical shock by exposed electrical light sockets and wires. Findings include: 1. During a survey on 10/22/19, at 11:30am, of resident rooms throughout various parts of the facility, light covers were missing ends that incase the fluorescent lighting fixture. See picture (#1). 2. During a survey on 10/22/19, at 12:30pm, of the shower room the exposed light fixture in the ceiling above the shower head on the wall had no light cover on the light bulb. See picture (#16). 3. During an interview on 10/22/19, at 12:30pm, (Staff# A Camron Hargrove not have an answer for the missing light fittings.	1. Light covers were obtained and will be replaced. To be completed by 3/18/2020. 2. Light fixtures in the bathroom will be replaced by 3/11/2020. A running maintenance log will be created that lists deficiencies, date added and date completed. This will be kept in the manager's office to ensure maintenance is being completed. Construction has begun to renovate the building and keeping up with repairs will be a priority.

STATE OF WASHINGTON
DEPARTMENT OF HEALTH
PO Box 47874 • Olympia, Washington 98504-7874

April 1, 2020

Kristen Prentice
Sea Mar Community Health Center
10001 17th Pl S
Seattle, WA 98168

Subject: Inspection Number X2019-469

Dear Ms. Prentice,

The Washington State Department of Health conducted a inspection at Sea Mar Community Health Center. Your inspection was conducted on 10/22/2019. The Plan of Correction that was submitted was approved on 4/1/2020.

I sincerely appreciate your cooperation and hard work during the inspection process and look forward to working with you again in the future.

Sincerely,

Jennifer Ross

Jennifer Ross, LMHC
Behavioral Health Reviewer